Psychiatric coercion and violence:

ethical, legal and preventive aspects

Coerción psiquiátrica y violencia: aspectos éticos, legales y preventivos

Natalia K. Rzhevskaya[1] https://orcid.org/0000-0001-8804-7441, Viktor A. Ruzhenkov[1] https://orcid.org/0000-0003-4039-7519, Victoria V. Ruzhenkova[1] https://orcid.org/0000-0003-0991-2084, Inna S. Khamskaya[1] https://orcid.org/0000-0001-9003-6629, Uliana S. Moskvitina[1] https://orcid.org/0000-0002-2835-2759*
[1]Belgorod State University, Medical Institute, 301015, 85 Pobedy St., Belgorod, Russia
**Corresponding author: Victoria V. Ruzhenkova, Belgorod State University, Medical Institute, 301015, 85 Pobedy St., Belgorod, Russia, Tell. +79803236506*
E-mail: ruzhenkova@bsu.edu.ru
Received/Recibido: 06/28/2020 Accepted/Aceptado: 07/15/2020 Published/Publicado: 09/09/2020 **DOI: 10.5281/zenodo.4265434**

Abstract

Introduction. Violence and coercion cause harm not only to the physical health but also to the mental condition and social well-being; it makes an impact on the effectiveness of direct care, prevents a favorable therapeutic alliance.

Aim. The objective of this review was to provide an analysis of the ethical, legal, and preventive aspects of coercion and violence measures applied in a psychiatric hospital based on literature data.

Material and Methods. We analyze research articles, mainly over the last 5 years, related to the problem of coercion and violence against patients in psychiatric hospitals in different countries from computer databases, PubMed, Medical-Science, E-Library, Web of Science, Scopus.

Results and Discussion. The patients of psychiatric hospitals are subjected at every turn to coercion and violence by medical staff and come across difficulties in protecting their rights and legitimate interests. In conditions of a psychiatric hospital, coercion affects directly the process of hospitalization, delivery of psychopharmacotherapy, organizing a protective regimen in departments, rendering urgent medical aid for psychomotor agitation, and other conditions, applying measures of physical coercion and isolation, and realization other essential patients' rights. The absence of a clear legal framework, clinical criteria, and ignoring the principles of bioethics are accompanied by abuses by medical staff, the use of various forms of violence against mental health patients', breaking their essential rights to their bodily integrity, freedom of movement and privacy.

Conclusion. It is required to develop regulatory legal acts controlling the procedure of restriction of the rights of persons with mental disorders and medical criteria. A bioethical approach to psychiatric care delivery, the use of psycho-educational programs, and psychosocial rehabilitation promote to increase patients' compliance and an effective therapeutic alliance with a psychiatrist.

Keywords: psychiatric care, coercion, violence, measures of physical coercion, psychiatric patient rights, discrimination.

Resumen

Introducción. La violencia y la coerción causan daño no solo a la salud física, sino también a la condición mental y al bienestar social; tiene un impacto en la efectividad de la atención directa, impide una alianza terapéutica favorable.

Objetivo. Esta revisión tuvo como objetivo analizar los aspectos éticos, legales y preventivos de las medidas de coerción y violencia que se aplican en un hospital psiquiátrico con base en datos de la literatura.

Material y métodos. Analizamos artículos de investigación, principalmente de los últimos 5 años, relacionados con el problema de la coerción y la violencia contra pacientes en hospitales psiquiátricos en diferentes países desde bases de datos de informática, PubMed, Medical-Science, Elibrary, Web of Science, Scopus.

Resultados y discusión. Los pacientes de los hospitales psiquiátricos son sometidos, en todo momento, a coacción y violencia por parte del personal médico, y se encuentran con dificultades para proteger sus derechos e intereses legítimos. En condiciones de hospitalización psiquiátrica, la coerción afecta directamente el proceso de hospitalización, la administración de psicofarmacoterapia, la organización de un régimen de protección en los departamentos, la prestación de asistencia médica urgente para la agitación psicomotora y otras afecciones, la aplicación de medidas de coerción física y aislamiento, y la consecución de otros derechos esenciales de los pacientes. La ausencia de un marco legal claro, criterios clínicos e ignorar los principios de la bioética, van acompañados de abusos por parte del personal médico, el uso de diversas formas de violencia contra la salud mental de los pacientes, irrespetando sus derechos esenciales de integridad corporal, libertad de movimiento e intimidad.

Conclusión. Se requiere desarrollar actos legales regulatorios que controlen el procedimiento de restricción de los derechos de las personas con trastornos mentales y criterios médicos. Un enfoque bioético a la prestación de atención psiquiátrica, el uso de programas psicoeducativos y la rehabilitación psicosocial promueven el cumplimiento de los pacientes y una alianza terapéutica efectiva con un psiquiatra.

Palabras clave: atención psiquiátrica, coerción, violencia, medidas de coerción física, derechos de pacientes psiquiátricos, discriminación.

Introduction

According to the WHO[1], violence is «the use of physical force or power, threatened or actual, against oneself, another person, or against a group or community that either result in or has a high likelihood of resulting in injury, death, psychological harm, maldevelopment or deprivation». Makushkin et al.[2] define violence as any type of behavior that is intended to cause harm to physical health, psychiatric health, well-being, insult, and humiliation. Cruel treatment and violence include physical and psychological cruelty, sexual abuse, isolation, various types of exploitation, disparagement, restriction of legal rights and freedoms, psychopharmacotherapy with inadequate doses of drugs. Coercion is an act of an external mental, physical, material, or organizational impact on a person's consciousness, will, and behavior[3].

The basis of the biopsychosocial model of psychiatric care includes preserving the patient's personality and his socio-environmental adaptation, improving the quality of life, and respecting his rights and freedoms[4]. At the same time, the stay of people with mental disorders in a psychiatric hospital is fraught with various restrictions, coercion, and violence[5]. Due to several reasons, the rights of patients with mental disorders in many cases are violated in the social and domestic sphere, in the educational and working process during medical care is provided[6], including psychiatric care[7].

For the above-mentioned reasons, we were prompted to analyze the ethical, legal, and preventive aspects of coercion and violence measures applied in a psychiatric hospital based on literature data.

Material and Methods

We performed a literature review over the last five years in the databases, including PubMed, Medical-Science, Elibrary, Web of Science, Scopus., focusing on the problem of application of various forms of coercion and violence against mental health patients by medical staff in psychiatric hospitals in different countries. Mentioning the concept of "violence against the mental health patients" in keywords was the criterion for selecting articles. A total of 832 articles were found, categorized by country, and their data extracted.

Results and Discussion

Persons with mental disorders are often dangerous for themselves and the others, at which point the measures of physical restraint and other forms of restricting their rights are applied during psychiatric care is provided. Involuntary admission to a psychiatric hospital in a total number of hospitalizations is up to 8.6%[8], while the proportion of people who undergo coercive treatment in a psychiatric hospital is 27.6%[9]. The evidence indicates that the prevalence of involuntary admissions in different countries is determined by the legal characteristics and specifics of the ethical aspects of the psychiatric care in each state and makes up 3-38%[10]. According to the data of Makushkina[11], the rate of involuntary admissions to psychiatric hospitals in Russia is about 44.4 per 100 thousand people; the prevalence of involuntary admissions in Western Europe is from 17 to 46.8 per 100 thousand people in the population[12].

Among the patient population who were hospitalized involuntarily, men predominate at the age of around 43 years (63.5-67.2%), single status (61.9-80.5%), and unemployed (69.5%)[13]. Patients with schizophrenia is up to 40.6-60.9%, with mood disorders -32-34.8%, with organic mental disorders - about 19%[14,45]. In addition, the psychomotor agitation is up to 2.6 - 52% of cases among medical emergencies in a psychiatric hospital[15]. Among risk factors for the development of psychomotor agitation with destructive behavior are male sex, young age, schizophrenia-related disorders, and systematic intake of psychoactive drugs, involuntary admissions, rehospitalizations, and episodes of aggressive behavior in past medical history[16]. The measures of physical restraint are applied to such kinds of patients in 10-30% of cases during the emergency medical treatment is provided[17].

A disregard of the legitimate interests and patients' rights and their social ills is widespread among medical staff of the psychiatric service[18]. Also, the vague legal and clinical fundamentals of using measures for restricting the rights of persons with mental disabilities lead to many violations and abuses in this sphere[19]. Persons with mental disorders are forced to execute a consent to voluntary psychiatric hospitalization and medical intervention and are refused a request to review their medical records; they are also under 24/7 observation and they take medicine and receive treatment procedures under the close control[20]. In the psychiatric unit patients deal with the impossibility of free movement and intimacy, they are denied to discontinue medical treatment at any stage or because of an immediate release. Many patients note the impossibility of obtaining any information about the received medical treatment and the refusal of an attending physician to discuss the tactics of the therapy measures[21].

Involuntary admission to a psychiatric hospital has many areas of concern, which are related to its ethical, legal, clinical, and social aspects[22]. The balance of timeliness of medical care, the safety of the others, and the respect for the patient's rights is a problem in performing an involuntary admission, regardless of the socio-political establishment and the level of state economic development[23].

The participation of the patient himself in the legal trial, according to the norms of international law, is mandatory. However, it is not uncommon in practice of the psychiatric hospitals that the patient is not called to a court session, and the judgment is made based on medical records, which violates the human right to protect their legitimate interests[24].

The patients who are hospitalized involuntarily have a very negative perception of their freedom of movement restrictions and total control, physical restraint, and a compulsory drug administration[25], which often turns to be the subject of violations and abuse by medical staff, including its performance without any medical prescription[26].

Mental disorders are often accompanied by psychomotor agitation with aggressive tendencies, which calls for application of isolation, physical restraint measures, forced drug administration, whit the consequence of abuse of patient's rights to human dignity, physical integrity, and personal freedom[27,28]. In the practice of psychiatric hospitals, it is also not uncommon that psychopharmacological drugs are used in high doses as a way of punishment for patients with aggressive destructive behavior[29].

The application of physical restraint measures in a psychiatric hospital is regulated by article 30 of the Federal Law of the Russian Federation of July 2, 1992 No. 3185-1 "On Psychiatric Care and Guarantees of Citizens' Rights during Its Provision", and the requirements of this statutory document are generally violated by psychiatrists administering the measures of physical restraint to the patients who were hospitalized voluntarily[30].

Patients deal with rough treatment, threats, and unjustified use of physical force, coercion to work in the hospital department on the part of the medical staff[31]. Many people with mental disorders also noted that they were not able to contact hospital administration with a request to report about violations of their rights, or that the answers to their complaints were rather formal[32].

The patients in psychiatric hospitals experience various forms of indirect coercion by medical staff: they are involved in food delivery, loading and unloading operations and repair works, cleaning the premises of the psychiatric unit and the hospital site, monitoring and caring for other patients[33]. About 80% of people with mental disorders complained of a lack of autonomy and freedom of action in the ward, they also negatively behave towards the constant control by medical staff[34], including during the performing of personal hygiene - 25%[35]. Patients also complain of the impossibility of personal visits with family members and private phone calls that are carried out only in the presence of medical staff or are prohibited completely[36,46].

The main reasons that lead to the discrimination of psychiatric patients are insufficiently clear legal fundamentals for restrictions of rights and freedoms of people with mental disorders by medical staff, an extremely simplified case analysis, which require its application, low legal competence, and poor knowledge of bioethics principles among medical staff in psychiatric hospitals[37]. Coercion in any form, which is applied in a psychiatric hospital, reduces significantly patients' compliance until they completely abandon the treatment, and also increases the frequency and severity of aggravation of psychopathological symptoms[38].

In the case of involuntary admission, before the trial, a person has already been in a psychiatric hospital for some time and has been deprived of freedom of movement, which is similar to imprisonment and that's why it also requires statutory regulation[39]. Involuntary admission to a psychiatric hospital requires of all the participants of this process to have interdisciplinary knowledge: the judiciary must have certain knowledge in the psychiatric area to analyze the medical part of the process and the psychiatrists must have legal competence to make decisions that lead to the legal consequences for their patients[40].

In providing psychiatric care patients' rights restriction requires clear legal and clinical regulation, including the establishment of principles of legal and social protection for medical staff and patients themselves[41].

The main objective of bioethics in providing psychiatric care is to develop tolerance and an open-minded attitude of psychiatrists and medical staff to people with mental disorders. On the other hand, it is necessary to limit coercive measures to the level of clinical necessity and expediency[42].

A differentiated approach to the application of the restrictive measures and the discussion of the treatment policy with the patient, when he is offered a choice of physical restraint, the drugs' administration or isolation in an individual ward, or 24-hour observation, gives a positive therapeutic effect, increases the level of trust to the attending physician and patient's compliance[43].

At the present stage of psychiatry and society development, the main aim of the psychosocial therapy and rehabilitation should consist in the consensus achievement between the psychiatrist and the patient on the issues of treatment, compulsory use of the psychotherapeutic approach, destigmatization of not only the patient himself but also his milieu and medical staff of psychiatric service[44]. As it is prescribed in Article 27 of the Federal Law of the Russian Federation of November 21, 2011, No. 323-FZ "On fundamental healthcare principles in the Russian Federation", the development of a responsible attitude of persons with mental disorders to their state of health is also within the competence of psychiatrists. It should develop policies and treatment approaches designed to respect patients' autonomy and reducing institutional coercion. Future research should focus on mental healthcare policies targeted at empowering treatment approaches, respecting the patient's autonomy, and promoting reductions of institutional coercion.

Conclusion

The evidence indicates that the legal framework specifying the powers of psychiatrists in the course of the restricting the rights of persons with mental disorders is not developed sufficiently at present; there are no medical criteria for the application of restrictive measures, which lead to abuse in this field. Prevention of coercion and violence in a psychiatric hospital should be based on the development of a contract (bioethical) approach to the interaction in the "patient-medical staff" system, and the development of a respectful attitude of

the medical community to their patients. On the other hand, the transition from the paternalistic model, which is traditionally used in psychiatric practice, to the bioethical (contract) one will increase the level of patients' responsibility for their health, their behavior, and create a sufficient level of compliance in a friendly therapeutic environment. The psychoeducational approach, which complies with the bioethical concept of psychiatric care, also promotes positive contact with patients, increasing his interest in cooperation with psychiatrists and medication adherence.

References

1. Krug, E.G., Dahlberg, L.L., Mercy, J.A., Zwi A.B., Lozano R., eds., 2002. World report on violence and health. Geneva, World Health Organization, 360 p.

2. Makushkin, E.V., Polishchuk, Y.I., Runikhina, N.K., Panchenko, E.A., 2017. Identification and prevention of harsh treatment of elderly and elderly people in specialized institutions of health and social protection. Assistance to victims of abuse: Methodical recommendations. Moscow: FSBU «NRMC named by V.P.Serbsky», 23 p. (In Russian)

3. Kovaleva, O.N., Yakovitsky, O.C., Kosenko, A.A., 2014. Legal regulation of the implementation of administrative enforcement measures. Mogilev: Mogilev higher College of the Ministry of Internal Affairs rep. Belarus. 88 p. (In Russian)

4. Minoletti, A., Toro, O., Alvarado, R., Carniglia, C., Guajardo, A., Rayo, X., 2015. A survey about quality of care and users' rights in Chilean psychiatric services. Revista Médica de Chile, 143(12), 1585–1592. DOI:10.4067/S0034-98872015001200012

5. Barnicot, K., Insua-Summerhayes, B., Plummer, E., Hart, A., Barker, C., Priebe, S., 2017. Staff and patient experiences of decision-making about continuous observation in psychiatric hospitals. Social Psychiatry and Psychiatric Epidemiology, 52(4):473-483. DOI:10.1007/s00127-017-1338-4

6. Nyhuis, P.W., Zinkler, M., 2019. Open-door psychiatry and community mental health work. Der Nervenarzt, 90(7):695-699. DOI: 10.1007/s00115-019-0744-0.

7. Walker, A.M., Klein, M.S., Hemmens, C., Stohr, M.K., Burton, V.S. Jr., 2016. The Consequences of Official Labels: An Examination of the Rights Lost by the Mentally Ill and Mentally Incompetent Since 1989. Community Ment Health Journal, 52(3):272-280. DOI:10.1007/s10597-015-9941-x

8. Demcheva, N.K., Yazdovskaya, A.V., Sidoryuk, O.V., 2015. Epidemiological indicators and performance measures for mental health services in the Russian Federation (2005-2013): Statistical Handbook. Moscow: FSBU «NRMC named by V.P.Serbsky», 572 p. (In Russian)

9. Gurovich, I.Y., Shmukler, A.B., Kostyuk, G.P., Naryshkin, A.V., 2013. Contingent of psychiatric hospital patients (based on the materials of a one-day census). Social and clinical psychiatry, 2:5-14. (In Russian)

10. Gilhooley, J., Umama-Agada, E., Asghar, M., McManus, S., Whitty, P.F., Kelly, B.D., 2017. Voluntary and involuntary psychiatric admissions in a suburban area: comparison with national rates, diagnosis and other correlates of involuntary admission status. Irish Journal of Psychological Medicine, 34(4):243-249. DOI: 10.1017/ipm.2017.44.

11. Makushkina, O.A., 2015. System of prevention of dangerous actions of mentally ill people in the Russian Federation: monitoring indicators and evaluating effectiveness. Russian psychiatric journal, 5:29-37. (In Russian)

12. Silva, B., Golay, P., Morandi, S., 2018. Factors associated with involuntary hospitalization for psychiatric patients in Switzerland: a retrospective study. BMC Psychiatry, 18(1):401. DOI: 10.1186/s12888-018-1966-6.

13. McGuinness, D., Murphy, K., Bainbridge, E., Brosnan, L., Keys, M., Felzmann, H., Hallahan, B., McDonald, C., Higgins, A., 2018. Individuals' experiences of involuntary admissions and preserving control: qualitative study. BJPsych Open, 16;4(6):501-509. DOI: 10.1192/bjo.2018.59.

14. Schmitz-Buhl, M., Gairing, S.K., Rietz, C., Häussermann, P., Zielasek, .J, Gouzoulis-Mayfrank, E.,2019. A retrospective analysis of determinants of involuntary psychiatric in-patient treatment. BMC Psychiatry, 29;19(1):127. DOI: 10.1186/s12888-019-2096-5.

15. Baldaçara, L., Ismael, F., Leite, V., Pereira, L.A., dos Santos, R.M., Gomes Júnior, V.P., Calfat, E.L., Diaz, A.P., Périco, C.D., Porto, D.M., Zacharias, C.E., Cordeiro, Q., Silva, A.G., Tung, T.C. ,2019. Brazilian guidelines for the management of psychomotor agitation. Part 1. Non-pharmacological approach. Brazilian Journal of Psychiatry, 41(2):153-167. DOI: 10.1590/1516-4446-2018-0163

16. Iozzino, L., Ferrari, C., Large, M., Nielssen, O., de Girolamo, G., 2015. Prevalence and Risk Factors of Violence by Psychiatric Acute Inpatients: A Systematic Review and Meta-Analysis. PLoS One, 10(6):e0128536. DOI: 10.1371/journal.pone.0128536

17. Gaynes, B.N., Brown, C.L., Lux, L.J., Brownley, K.A., Van Dorn, R.A., Edlund, M.J., Coker-Schwimmer, E., Weber, R.P., Sheitman, B., Zarzar, T., Viswanathan, M., Lohr, K.N. 2017. Preventing and De-escalating Aggressive Behavior Among Adult Psychiatric Patients: A Systematic Review of the Evidence. Psychiatric Services, 1;68(8):819-831. DOI: 10.1176/appi.ps.201600314

18. Verbeke, E., Vanheule, S., Cauwe, J., Truijens, F., Froyen, B., 2019. Coercion and power in psychiatry: A qualitative study with ex-patients. Social Science and Medicine, 223:89-96. DOI: 10.1016/j.socscimed.2019.01.031

19. Gouzoulis-Mayfrank, E., Gairing, S.K., Krämer, T., Förster, M., Schmitz-Buhl, M., 2019. Öffnung einer psychiatrischen Akutstation als komplexe Intervention [Opening up an acute psychiatric ward as a complex intervention]. Nervenarzt, 90(7):709-714. DOI:10.1007/s00115-019-0734-2

20. Berring, L.L., Pedersen, L., Buus, N., 2016. Coping with Violence in Mental Health Care Settings: Patient and Staff Member Perspectives on De-escalation Practices. Archives of psychiatric nursing, 30(5):499-507. DOI: 10.1016/j.apnu.2016.05.005

21. Wyder, M., Bland, R., Crompton, D., 2016. The importance of safety, agency and control during involuntary mental health admissions. Journal of Mental Health, 25(4):338-342. DOI:10.3109/09638237.2015.1124388

22. Verbruggen-Smulders, C.H.C.M., Postulart, D., Voskes, Y., 2019. High and intensive care in long-term inpatient psychiatric services. Tijdschrift voor psychiatrie, 61(5):317-325.

23. Raveesh, B.N., Lepping, P., 2019. Restraint guidelines for mental health services in India. Indian Journal of Psychiatry, 61(4):698-705.

24. Markiewicz, I., Heitzman, J., Gardyńska-Ziemba, E., 2016. Involuntary psychiatric holds – the structure of admissions on the example of Institute of Psychiatry and Neurology in Warsaw. Psychiatria polska, 50(1):7-18. DOI: 10.12740/PP/33336.

25. Bottlender, R., Juckel, G., 2019. Coercion and aggression in psychiatry: the individual psychological dimension of aggressive and

coercive acts by therapists. Fortschritte der Neurologie Psychiatrie, 87(10):540-547. DOI: 10.1055/a-0597-1974.

26. Molewijk, B., Kok, A., Husum, T., Pedersen, R., Aasland, O., 2017. Staff's normative attitudes towards coercion: the role of moral doubt and professional context-a cross-sectional survey study. BMC Med Ethics, 18(1):37. DOI:10.1186/s12910-017-0190-0

27. Sampogna, G., Luciano, M., Del Vecchio, V., Pocai, B., Palummo, C., Fico, G., Giallonardo, V., De Rosa, C., Fiorillo, A., 2019 Perceived Coercion Among Patients Admitted in Psychiatric Wards: Italian Results of the EUNOMIA Study. Front Psychiatry, 10:316. DOI: 10.3389/fpsyt.2019.00316

28. Rzhevskaya, N.K., Ruzhenkov, V.A., Boeva, A.V., Lukyantseva, I.S., 2019. Incorrect psychopharmacotherapy as a cause of applications of physical restraint measures to patients, hospitalized in a psychiatric hospital on a voluntary basis. Drug Invention Today, 12(9):2089-2093

29. Polischuk, Y.I., Letnikova, Z.V., 2018. Abuse of elderly mental patients in families and in psychiatric facilities. Social and clinical psychiatry, 1:101-105 (In Russian)

30. Rzhevskaya, N.K., Ruzhenkov, V.A., 2018. Measures of physical restraint in the provision of psychiatric care (epidemiological, clinical and social aspects). Mental health, 16(9):21-31. (In Russian) DOI: 10.25557/2074-014X/2018.09.21-31.

31. Hamid, A.Y.S.M., Daulima, C.N.H., 2018. The experience of restraint-use among patients with violent behaviors in mental health hospital. Enfermería Clínica, 28(1):295-299. DOI: 10.1016/S1130-8621(18)30173-6

32. Pollmächer, T., 2019. Autonomy focusing as guiding idea of minimally restrictive psychiatry. Nervenarzt, 90(7):669-674. DOI:10.1007/s00115-019-0714-6

33. Staniszewska, S., Mockford, C., Chadburn, G., Fenton, S.J., Bhui, K., Larkin, M., Newton, E., Crepaz-Keay, D., Griffiths, F., Weich, S., 2019. Experiences of in-patient mental health services: systematic review. British Journal of Psychiatry, 214(6):329-338. DOI: 10.1192/bjp.2019.22.

34. Woodward, S., Berry, K., Bucci, S., 2017. A systematic review of factors associated with service user satisfaction with psychiatric inpatient services. Journal of Psychiatry Research, 92:81-93. DOI: 10.1016/j.jpsychires.2017.03.020

35. Kruchinin, Y.S., 2015. Special report of the Commissioner for human rights in the Chuvash Republic "on the observance of the rights of citizens held in psychiatric hospitals and psychoneurological boarding schools of the Chuvash Republic", 73 p. (In Russian)

36. Kerbage, H., El Chammay, R., Richa, S., 2016. Mental health legislation in Lebanon: Nonconformity to international standards and clinical dilemmas in psychiatric practice. International Journal of Law and Psychiatry, 44:48-53. DOI: 10.1016/j.ijlp.2015.08.031

37. Gowda, G.S., Lepping, P., Ray, S., Noorthoorn, E., Nanjegowda, R.B., Kumar, C.N., Math, S.B., 2019. Clinician attitude and perspective on the use of coercive measures in clinical practice from tertiary care mental health establishment - A cross-sectional study. Indian Journal of Psychiatry, 61(2):151-155.

38. Al Saif, F., Al Shakhoori, H., Nooh, S., Jahrami, H., 2019. Association between attitudes of stigma toward mental illness and attitudes toward adoption of evidence-based practice within health care providers in Bahrain. PLoS ONE, 14(12):e0225738. DOI: 10.1371/journal.pone.0225738

39. Galkovskaya, N.G., 2015. Problems of improving the mechanism of judicial protection of the rights of citizens hospitalized in a medical organization that provides psychiatric care in inpatient conditions, in an involuntary manner. Bulletin of Tomsk state Universit, 3(17):9-94. (In Russian)

40. Demeneva, A.V., 2014. Legal consequences of rulings of the European court of human rights in cases of psychiatric care in Russia. International justice, 2:21-43. (In Russian)

41. Alevizopoulos, G., Bozikas, V., Touloumis, C., 2017. Restraints in Greek mental health services: Ethics, practice and costs. Psychiatriki, 28(4):306-313. doi:10.22365/jpsych.2017.284.306

42. Shvets, K.N., Khamskaya, I.S., 2019. Factors of social disadaptation of patients with schizophrenia and approaches to psychosocial therapy and rehabilitation (review). Research Results in Biomedicine, 5 (2): 72-85 (In Russian).

43. Mielau, J., Altunbay, J., Heinz, A., Reuter, B., Bermpohl, F., Rentzsch, J., Lehmann, A.I., Montag, C., 2017. Coercive Interventions in Psychiatry: Prevention and Patients' Preferences]. Psychiatr Prax, 44(6):316-322. DOI:10.1055/s-0042-105861

44. Krieger, E., Moritz, S., Weil, R., Nagel, M., 2018. Patients' attitudes towards and acceptance of coercion in psychiatry. Psychiatry Research, 260:478-485. DOI:10.1016/j.psychres.2017.12.029

45. Majumder, P., Biswas, P., & Majumder, S. (2020). Application of New TOPSIS Approach to Identify the Most Significant Risk Factor and Continuous Monitoring of Death of COVID-19. Electron J Gen Med. 2020; 17 (6): em234.

46. Vidinov, K. N., & Stoinov, Y. (2019). Do we really Need Lugol Solution in the Era of Energy Devices for the Preoperative Management of Patients with Graves' Disease?. *Journal of Clinical and Experimental Investigations*, *10*(4), em00730.